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Northgate Valley Clinic

Radiology Report - CT Abdomen and Pelvis with Contrast

Patient: Anneke Rumbaugh-Tsai | **DOB:** 1983-06-27 | **MRN:** NVC-5582137 | **Study date:** 2026-07-19 | **Accession:** CT-2026-0719-0442 | **Reporting radiologist:** Dr. Halvard Mbeki

Clinical indication

Four days of right iliac fossa pain with anorexia and a low-grade fever. Equivocal ultrasound earlier the same day in which the appendix was not visualised. Rule out appendicitis and other causes of right lower quadrant pain.

Technique

Multidetector CT of the abdomen and pelvis in the portal venous phase following 90 mL of intravenous iodinated contrast. Axial acquisition at 1 mm collimation with coronal and sagittal reformats. No oral or rectal contrast was given. Dose length product 486 mGy.cm.

Findings

The appendix is identified in a retrocaecal position and is dilated to 11 mm with wall thickening, mural hyperenhancement and surrounding fat stranding. A 4 mm calcified appendicolith sits at the base. There is a small volume of free fluid in the pelvis and several reactive right iliac nodes up to 9 mm in short axis. No organised collection and no extraluminal gas to suggest perforation. The terminal ileum and caecum are normal in calibre with no wall thickening. Liver, spleen, pancreas, adrenal glands and both kidneys are unremarkable, with no biliary or urinary tract dilatation and no renal calculi. The gallbladder is thin walled and contains no calculi. Bowel loops elsewhere are normal with no obstruction. The uterus and both ovaries are within normal limits and there is no adnexal mass. No abdominal aortic aneurysm; the aorta and its main branches opacify normally. Lung bases show minor dependent atelectasis and no effusion. No lytic or sclerotic bone lesion is seen. Incidental note is made of a 9 mm simple cyst in the lower pole of the left kidney, which needs no follow-up.

Impression

Acute uncomplicated appendicitis with an appendicolith and a small volume of reactive pelvic free fluid. No evidence of perforation or abscess. No alternative cause for the pain.

Communication

Findings were telephoned to the on-call surgical registrar at 14:52 on the day of the study and acknowledged. Report verified electronically by the reporting radiologist.